



THERAPEUTIC Newsletter

An official Publication of
The Department of Pharmacy Practice, Chalapathi Institute of Pharmaceutical Sciences, GUNTUR- 34.
(AUTONOMOUS)

Accredited with 'A' Grade by NAAC

Phone : 0863-2524124,125, Email: chalapathidic@gmail.com : principalcips@gmail.com

Volume - IX

Issue -II

Apr - June- 2016

Mission : Empower students with professional knowledge and competence, ethics and pharmaceutical services.

Vision : Endeavour pharmaceutical services for the purpose of achieving optimal medication therapy and improving patient's quality of life.

From Editor's desk

Welcome to the second issue (Vol.IX) of therapeutic newsletter published by the Department of Pharmacy Practice, Chalapathi Institute of Pharmaceutical Sciences, Guntur. This newsletter brings you the updated information of our departmental activities and also to update the knowledge through physician desk .

NATIONAL WORKSHOP ON PHARMACOVIGILANCE 23rd APRIL 2016 ORGANIZED BY DEPT. OF PHARMACY PRACTICE, CLPT, WITH TECHNICAL SUPPORT OF PvPI



GUEST LECTURES ARRANGED BY THE DEPARTMENT

S.No	GUEST	TOPIC NAME	DATE
01.	G. Bhanu Prathap	Clinirex Database	18-04-2016
02.	Dr. V. Kalaiselvan	Pharmacovigilance Programme of India	23-04-2016
03.	T. Viswanath	Career Opportunities In Pv Industry	23-04-2016
04.	Dr. Parthiban Manimaran	Pharmacovigilance Work Profile In Industry	23-04-2016
05.	Dr. Meena Kumari & Mrs. N. Lakshmi Prasanthi	Hands On Training On Adr Reporting And Causality Assessment	23-04-2016
06.	Dr. Adepu Ramesh	Internship review	11-06-2016
07.	T. Viswanath	Pharmacovigilance	05-06-2016

PUBLICATIONS:-

Chellangi Sindhuja 1, *, Pothina Sushma Chowdary1 , Paritala Manohar1 , K Md Umar 2 , B Sailaja 3 published titled as "Measurement of Outcomes in Hypertensive Patients with Relation to Counselling" in International Journal of Pharma Research and Health Sciences, Volume 4 (3), 2016, Page-1202-1209

Editorial board:

Chief Patron :

Sri. Y.V.Anjaneyulu

President, Chalapathi Educational Society

Patron :

Dr. D.S.Raju Naidu, M.D., DM

Medical Superintendent, GGH, Guntur

Dr. Nadendla Rama Rao,

Principal, Chalapathi Institute of Pharmaceutical Sciences

Dr. N.Murali Krishna, M.D

Prof. & Head, Department of Psychiatry, GGH, Guntur

Dr. Mohan Rao, M.D

Prof. & Head of General Medicine, GGH, Guntur

Dr. M.S. Raju, M.D

Prof. & Head, Department of Pediatrics, GGH, Guntur

Chief Editor:

Sri. N.V.Rama Rao

Executive Editors:

Dr.K.Nani Babu

Smt.P. Sharmila Nirojini

Smt.Sk.Shafiya Begum

Dr.R.Venkateswar Rao

Dr.M.Maneesh Kumar

Student Editors:

T.Poorna Chand

Y.Divya

M.Joshna Sri

B.Srinivas Teja

CONFERENCES ATTENDED:

- N.V.Rama Rao, M. Maneesh Kumar, V/VI & VI/VI pharm.d students have participated and presented oral & poster presentations in **National Conference on Pharmacy Practice & Education System in India, Kerala on 07-04-2016 to 08-04-2016.**
- Smt. P. Sharmila Nirojini and Dr. Nani Babu Kalla has attended **II nd convention of IACP, Coimbatore, India on 2nd & 3rd April 2016.**
- B.Srinivas Teja, V/VI Pharm D Student has participated in MANTHAN 2016: ALL INDIA ESSAY CONTEST which was held at IIHMR UNIVERSITY, SCHOOL OF PHARMACEUTICAL MANAGEMENT, and JAIPUR, India on
- B.Srinivas Teja, V/VI Pharm D student has participated and presented a poster at 4th International Conference on Pharmacogenomics, Proteomics in Drug Discovery, Development and Clinical Therapeutics on 13/14-04-2016 at Chitkara University in collaboration with Harvard Medical School, USA.

ACHIEVEMENTS:

- VI/VI Pharm.D student, M.Sai Priya presented a poster presentation on “Branded Vs Generic Medicines – A Comparative Evaluation of Price and Quality of the Same Manufacture in India” and secured I Prize in **National Conference on Pharmacy Practice & Education System in India, Kerala on 07-04-2016 to 08-04-2016.**
- V/VI Pharm.D student, B.Chaithanya Lakshmi presented a oral presentation on “Clinical Pharmacist (Psychiatry) Role In Pharmacovigilance” and secured I Prize in **National Conference on Pharmacy Practice & Education System in India, Kerala on 07-04-2016 to 08-04-2016.**
- VI/VI Pharm.D student, K.Sipora presented a poster presentation on “Pharmacoeconomics Study of Oral Anti Diabetic Drugs Available In Andhra Pradesh And Indian Market” and secured II Prize in **National Conference on Pharmacy Practice & Education System in India, Kerala on 07-04-2016 to 08-04-2016.**

ACTIVITIES PERFORMED:

- National Workshop on Pharmacovigilance **23rd April 2016** Organized by Dept. of Pharmacy Practice, Chalapathi Institute of Pharmaceutical Sciences, with technical support of PvPI.
- Hands on training on Definitions and Standards for Expedited Reporting on **05-06-2016** by Dept. of Pharmacy Practice, at Chalapathi Institute of Pharmaceutical Sciences, Guntur.

PHYSICIAN DESK -



Dr.Rama Krishna

Asst.Professor
Dept of Neurology,
Govt. General Hospital, Guntur.

Parkinson's Disease

Named after James Parkinson who published 'An Essay on the Shaking Palsy' in 1817, which established Parkinson's as a recognised medical condition.

He studied at the London Hospital Medical College, qualifying as a surgeon in 1784 when he was 29. Parkinson's disease (PD) is a chronic and progressive movement disorder, meaning that symptoms continue and worsen over time. Nearly one million people in the US are living with Parkinson's disease. The cause is unknown, and although there is presently no cure, there are treatment options such as medication and surgery to manage its symptoms.

Major Symptoms-TRAP : Tremor, Rigidity, Akinesia/Bradykinesia, Postural Instability

Other Motor Symptoms Include : Gait, Dystonia, Hypophonia, Drooling, Dysphagia, Fatigue, Akathisia, Nonmotor Symptoms, Mood: 20-80% suffer from depression., Behavior: indirectly, e.g., a result of dementia, depression., Thinking: slowed reaction time and executive dysfunction, Sensation: impaired sense of smell, Excessive daytime sleep, insomnia, and REM sleep disturbances, Vision problems, Impaired proprioception, Drooling

Primary Known Causes

- Idiopathic: majority of cases, Genetic, Drug induced: Calcium Channel Blockers, Toxins: Supported by the geographically varied incidence, Head Trauma, Cerebral Anoxia

Pathophysiology:

- Decreased stimulation of the motor cortex by the basal ganglia, usually due to the inadequate production and action of dopamine (produced in the dopaminergic neurons of the brain.)
- The specific region affected seems to be the pars compacta in the substantia nigra where there is a marked loss in dopaminergic cells.
- We also see a considerably high activity in the cells of the Subthalamic nucleus, which inhibits movement.
- High presence of Lewy bodies in dopaminergic cells.

Diagnosis:

- PET Scan—decreased dopaminergic activity in the substantia nigra
- Unified Parkinsons Disease Rating Scale—cognitive interview
- Normal CT, Normal MR

Treatment:

Oral Medication

- L-Dopa (aka Levodopa) Most popular treatment Form of dopamine which is able to cross the BBB through transport in L-AA system and can then be metabolized to dopamine.
- **Sinemet** : levodopa + carbidopa
- **MAO-B Inhibitors**
- **Selegiline** : most common
- **Dopamine Agonists**: Ropinirole , Apomorphine , Lisuride
- **COMT Inhibitors** (Catachol-O-methyl transferase Inh.): Tolcapone Entacapone **Stalevo**: levodopa, carbadopa, and entacapone

Surgical Procedures

- Deep Brain Stimulation
- Creating a lesion in the sub-thalamic nucleus or globus pallidus .

STUDENT DESK -



B.SRINIVAS TEJA

Doctor of Pharmacy (Intern)
CLPT, Guntur.

Wolff-Parkinson-White Syndrome

Wolff-Parkinson-White syndrome is a heart condition in which there is an abnormal extra electrical pathway of the heart. The condition can lead to episodes of rapid heart rate (tachycardia).

Wolff-Parkinson-White syndrome is one of the most common causes of fast heart rate disorders in infants and children.

Causes

Normally, electrical signals in the heart go through a certain pathway that helps the heart beat regularly. The wiring of the heart prevents extra beats from occurring and keeps the next beat from happening too soon.

In people with Wolff-Parkinson-White syndrome, some of the heart's electrical signal goes down an extra (accessory) pathway. This may cause a very rapid heart rate called supraventricular tachycardia.

Most people with Wolff-Parkinson-White syndrome do not have any other heart problems. However, this condition has been linked with other conditions, such as Ebstein's anomaly. There is also a form that runs in families.

Symptoms

A person with this syndrome may have:

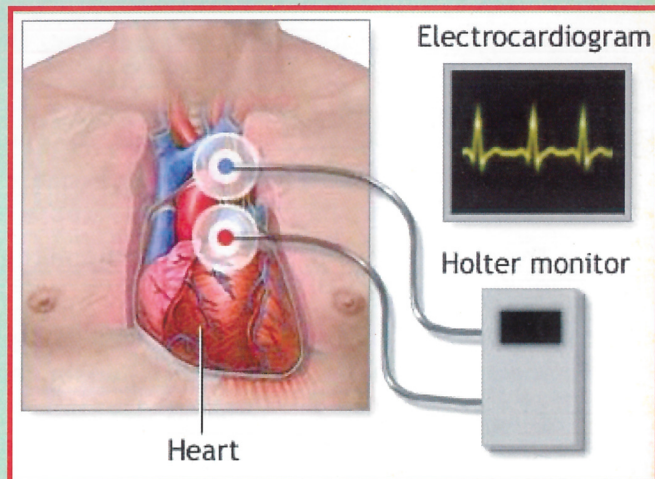
- Chest pain or chest tightness
- Dizziness
- Light-headedness
- Fainting
- Palpitations (a sensation of feeling your heart beat)
- Shortness of breath

Exams and Tests

An exam performed during a tachycardia episode will show a heart rate faster than 100 beats per minute. A normal heart rate is 60 - 100 beats per minute in adults, and under 150 beats per minute in newborns, infants, and small children. Blood pressure will be normal or low.

If the patient is currently not having tachycardia, the physical exam may be completely normal.

Wolff-Parkinson-White syndrome may be diagnosed through continuous ambulatory ECG monitoring, such as with a Holter monitor. A test called an electrophysiologic study (EPS) is done using catheters that are threaded up to the heart from an IV placed in the leg. It may help identify the location of the extra electrical pathway.



Treatment

Medicine such as adenosine, antiarrhythmic drugs, and amiodarone may be used to control or prevent a rapid heartbeat. If the heart rate does not return to normal with medication, doctors may use a type of therapy called electrical cardioversion (shock).

The long-term treatment for Wolff-Parkinson-White syndrome is catheter ablation. This procedure involves inserting a tube (catheter) into an artery through a small cut near the groin up to the heart area. When the tip reaches the heart, the small area that is causing the fast heart rate is destroyed using a special type of energy called radiofrequency.

Open heart surgery to burn or freeze the extra pathway may also provide a permanent cure for Wolff-Parkinson-White syndrome. However, surgery is usually done only if they need surgery for other reasons.

NATIONAL WORKSHOP ON PHARMACOVIGILANCE 23rd APRIL 2016



Book-Post



All Suggestions and comments may please be sent to:
The Editorial Board
Department of Pharmacy Practice

THERAPEUTIC *Newsletter*

Chalapathi Institute of Pharmaceutical Sciences

(AUTONOMOUS)

Accredited with 'A' Grade by NAAC

Chalapathi Nagar, Lam, GUNTUR-522034, Ph:0863-2524124 /5, Cell :9440101685,
Email : chalapathidic@gmail.com : Website : www.chalapathipharma.in